

KING FAHD CENTRAL HOSPITAL (KFCH)

Comprehensive Stroke Program

ANNUAL STROKE PROGRAM EVALUATION

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Prepared Date	_____	Review Date	_____	Reporting Period	_____ to _____

Purpose: Provide a structured annual assessment of the KFCH Comprehensive Stroke Program, integrating program volume, timeliness, clinical outcomes, quality measures, safety, education, EMS/system-of-care performance, patient experience, policy review, and improvement priorities.

1. Executive Program Summary

Overall annual program assessment	☐ Meets objectives ☐ Partially meets objectives ☐ Improvement required
Major achievements	_____
Major gaps / risks	_____
Significant service changes	_____
Accreditation/readiness issues	_____
Top priorities for next year	_____

2. Stroke Program Volume & Case Mix

Indicator	Annual Result	Prior Year	Trend	Comments
Total stroke/TIA encounters			☐ ↑ ☐ → ☐ ↓	
Acute ischemic stroke			☐ ↑ ☐ → ☐ ↓	
Intracerebral hemorrhage			☐ ↑ ☐ → ☐ ↓	
Subarachnoid hemorrhage			☐ ↑ ☐ → ☐ ↓	
TIA			☐ ↑ ☐ → ☐ ↓	
IV thrombolysis cases			☐ ↑ ☐ → ☐ ↓	
Mechanical thrombectomy cases			☐ ↑ ☐ → ☐ ↓	
Transfers in			☐ ↑ ☐ → ☐ ↓	
Transfers out			☐ ↑ ☐ → ☐ ↓	

3. Time-Critical Performance & Quality Measures

Measure	Target	Annual Result	Prior Year	Status	Action / Comment
Door-to-physician / initial evaluation				☐ Met ☐ Partial ☐ Gap	
Door-to-CT initiation				☐ Met ☐ Partial ☐ Gap	
CT interpretation / imaging turnaround				☐ Met ☐ Partial ☐ Gap	
Door-to-needle for IV thrombolysis				☐ Met ☐ Partial ☐ Gap	
Door-to-groin / arterial puncture				☐ Met ☐ Partial ☐ Gap	
Door-to-reperfusion				☐ Met ☐ Partial ☐ Gap	
Dysphagia screening compliance				☐ Met ☐ Partial ☐ Gap	
NIHSS documentation compliance				☐ Met ☐ Partial ☐ Gap	
VTE prophylaxis compliance				☐ Met ☐ Partial ☐ Gap	
IV thrombolysis utilization				☐ Met ☐ Partial ☐ Gap	
Mechanical thrombectomy utilization				☐ Met ☐ Partial ☐ Gap	
Transfer turnaround / DIDO when applicable				☐ Met ☐ Partial ☐ Gap	

4. Clinical Outcomes & Patient Safety

Outcome / Safety Indicator	Annual Result	Prior Year	Trend	Review / Action
In-hospital mortality			☐ Better ☐ Stable ☐ Worse	
Symptomatic intracranial hemorrhage after thrombolysis			☐ Better ☐ Stable ☐ Worse	



Procedure-related EVT complications			☐ Better ☐ Stable ☐ Worse	
Stroke Unit length of stay			☐ Better ☐ Stable ☐ Worse	
ICU utilization / LOS			☐ Better ☐ Stable ☐ Worse	
Discharge mRS / functional outcome			☐ Better ☐ Stable ☐ Worse	
90-day mRS when collected			☐ Better ☐ Stable ☐ Worse	
Readmission			☐ Better ☐ Stable ☐ Worse	
Falls / pressure injuries / other relevant safety events			☐ Better ☐ Stable ☐ Worse	

5. Stroke Unit, Rehabilitation & Continuity of Care

Domain	Annual Assessment	Evidence Reviewed	Gap / Improvement
Stroke Unit admission/discharge processes	☐ Effective ☐ Partial ☐ Gap		
Neurological monitoring	☐ Effective ☐ Partial ☐ Gap		
Swallow screening and nutrition	☐ Effective ☐ Partial ☐ Gap		
Early mobilization	☐ Effective ☐ Partial ☐ Gap		
PT/OT/SLP assessment and referral	☐ Effective ☐ Partial ☐ Gap		
Discharge readiness / education	☐ Effective ☐ Partial ☐ Gap		
Secondary prevention	☐ Effective ☐ Partial ☐ Gap		
Follow-up / continuity	☐ Effective ☐ Partial ☐ Gap		

6. EMS, Transfer Network & System of Care

Element	Annual Assessment	Evidence / Data	Action
EMS prehospital notification	☐ Effective ☐ Partial ☐ Gap		
Prehospital stroke identification / triage	☐ Effective ☐ Partial ☐ Gap		
Interfacility transfer performance	☐ Effective ☐ Partial ☐ Gap		
LVO/MT transfer process	☐ Effective ☐ Partial ☐ Gap		
Image/information sharing	☐ Effective ☐ Partial ☐ Gap		
EMS feedback / joint review	☐ Effective ☐ Partial ☐ Gap		
Referral-network coordination	☐ Effective ☐ Partial ☐ Gap		

7. Education, Competency & Community Awareness

Element	Planned	Completed	Evidence	Gap / Next Action
Annual staff stroke education				
NIHSS competency				
Stroke Unit nursing competency				
IV thrombolysis competency				
Post-EVT care competency				
Mock Stroke Code / simulation				
Patient/caregiver education				
Community stroke awareness activities				

8. Governance, Policies, Registry & Quality Improvement

Governance / QI Requirement	Status	Evidence	Required Action
Stroke Committee/IPC meetings and action follow-up	☐ Complete ☐ Partial ☐ Gap		
Annual policy/protocol review completed	☐ Complete ☐ Partial ☐ Gap		
Stroke registry completeness/accuracy review	☐ Complete ☐ Partial ☐ Gap		
KPI dashboard reviewed and reported to leadership	☐ Complete ☐ Partial ☐ Gap		
QI projects / corrective actions evaluated for effectiveness	☐ Complete ☐ Partial ☐ Gap		
Patient/family satisfaction reviewed	☐ Complete ☐ Partial ☐ Gap		



Adverse events / complications reviewed	☐ Complete ☐ Partial ☐ Gap		
Research / innovation activity reviewed when applicable	☐ Complete ☐ Partial ☐ Gap		

9. Annual Improvement Plan

Priority Gap	Root Cause / Rationale	Action	Owner	Due Date	Measure of Success	Status

10. Overall Conclusion & Governance Approval

Program effectiveness conclusion	
Resources / leadership support required	
Priorities approved for next annual cycle	
Evaluation presented to Stroke Committee / leadership	Date: _____ Minutes/Reference: _____

Stroke Coordinator	_____	Date	_____
Stroke Medical Director	_____	Date	_____
Quality Representative	_____	Date	_____
Executive / Authorized Approver	_____	Date	_____

Evidence to retain with this evaluation: annual KPI dashboard, registry summary, Stroke Committee/IPC minutes, policy-review log, QI project reports, complication/safety reviews, education and competency evidence, EMS/transfer data, patient/family satisfaction results, community-awareness evidence, and de-identified case samples where required.